Atezolizumab + nab-paclitaxel

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1 and 15 · atezolizumab + nab-paclitaxel

Active medicines: Atezolizumab + Nab-paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before each administered chemotherapy dose Review QT risk, constipation, liver function and any steroid already prescribed.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be confirmed Review contraindications, renal function and interactions.

Additional phase-specific support

- No routine corticosteroid or antihistamine premedication is required by the source for these medicines. Prior infusion reaction needs an individual plan.
- Give atezolizumab first, then nab-paclitaxel. Do not substitute solvent-based paclitaxel premedication.
- Review immune symptoms, count results and neuropathy before administration.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00688 v2a · p7 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.19 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Day 8 · nab-paclitaxel only

Active medicines: Nab-paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before each administered chemotherapy dose Review QT risk, constipation, liver function and any steroid already prescribed.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be confirmed Review contraindications, renal function and interactions.

Additional phase-specific support

- Atezolizumab is not scheduled on Day 8.
- No routine solvent-taxane steroid premedication. Check the current count and neuropathy decision.
- Continue to assess immune symptoms from previous atezolizumab even on chemotherapy-only days.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00688 v2a · p7 · NCCP antiemetics v8, Table 1 pp4–5

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Atezolizumab alone · after chemotherapy stops

Active medicines: Atezolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine chemotherapy antiemetic or nab-paclitaxel premedication.

- Keep the source thyroid/blood monitoring and immune-toxicity review; stopping chemotherapy does not end immune risk.
- Confirm continuation has been authorised and use the atezolizumab schedule in this source.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00688 v2a · p7 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.19 · Regimen-level drafts; prescriber and oncology-pharmacy verification required